TRUST | CARE | CURE

(A Unit of AS Kidney Hospital & Stone Centre Pvt. Ltd.)

Date	**17/02/2026**	**Srl No.**	**6**	**Patient Id**	**2602170006**
Name	**Mrs.**	**ge**	**71 Yrs.**	**Gender**	**Female**
Ref. By		**IPD No.**	**25/0927**	**UHID No.**	**P0005082**

COMPLETE BLOOD COUNT (CBC)

Test Name	Result	Unit	Normal Value
Haemoglobin (Hb)	**6.0**	gm/dl	12.0 - 15.0
Total Leucocyte Count (TLC)	**17,200**	/cumm	4000 - 11000
Differential Leucocyte Count (DLC)			
Neutrophil	**87**	%	40 - 75
Lymphocyte	**10**	%	20 - 45
Eosinophil	01	%	01 - 06
Monocyte	02	%	02 - 10
Basophil	00	%	0 - 01
Red Blood Cell (RBC)	**2.48**	million/µL	3.8 - 4.8
Packed Cell Volume (PCV/HCT)	**20.8**	%	37.0 - 48.0
Mean Corpuscular Volume(MCV)	84.1	fl	81.0 - 99.0
Mean Corpuscular Hemoglobin (MCH)	**24.1**	pg	27.0 - 31.0
MCHC	**28.8**	g/dl	32.0 - 36.0
Platelet Count (Plt Count)	2.23	Lakh	1.50 - 4.50

****** End Of Report ******

SR.LA ICIAN

M.B.B.S, M.D
CONSULTANT PATHOLOGIST
DMCI 10558

Date	17/02/2026	Srl No.	6	Patient Id	2602170006
Name	[redacted]	ge	71 Yrs.	Gender	Female
Ref. By	[redacted]	IPD No.	25/0927	UHID No.	P0005082

KIDNEY FUNCTION TEST (KFT)

Test Name	Result	Unit	Normal Value
Creatinine	0.79	mg%	0.60 - 1.20
Urea	32.0	mg /dl	13.0 - 45.0
Blood Urea Nitrogen (BUN)	14.95	mg/dl	6.0 - 21.0
Uric Acid	5.70	mg%	2.50 - 6.80
Calcium	9.30	mg/dl	8.0 - 10.5
Inorganic Phosphorus	4.25	mg/dl	2.50 - 4.50
Ionic Calcium	4.45	mg/dl	4.40 - 5.40
Total Protein	7.30	gm/dl	6.00 - 8.30
Albumin	4.40	gm/dl	3.20 - 5.00
Globulin	2.9	gm/dl	2.30 - 3.50
A/G Ratio	1.51		1.00 - 2.10

**** End Of Report ****

SR.LAB.TECHNICIAN

[redacted]
M.B.B.S, M.D
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DMCI 10558

Date	17/02/2026	Srl No.	6	Patient Id	2602170006
Name		ge	71 Yrs.	Gender	Female
Ref. By		IPD No.	25/0927	UHID No.	P0005082

HAEMATOLOGY

Test Name	Result	Unit	Normal Value
Blood Group ABO	"B"		
Rh Typing	POSITIVE		
HbA1c (Glycated hemoglobin)	6.30	%	4.00 - 6.00

Ref Range for HBA1c

Non Diabetic : < 5.7 %
Pre-Diabetic : 5.7 - 6.5 %
Diabetic : > 6.5 %

Remark :- Hemoglobin A1c criteria for diagnosing diabetes have not been established for patients who are <18 years of age.

HbA1c goals in treatment of diabetes :

Ages 0-6 years : 7.6% - 8.4%
Ages 6-12 years : <8%
Ages 13-19 years : <7.5%
Adults : <7%

COMMENT :-

The Glycosylated Hemoglobin (HbA1c or A1c) test evaluates the average amount of glucose in the blood over the last 2 to 3 months. This test is used to monitor treatment in someone who has been diagnosed with diabetes. It helps to evaluate how well the person's glucose levels have been controlled by treatment over time. This test may be used to screen for and diagnose diabetes or risk of developing diabetes. Depending on the type of diabetes that a person has, how well their diabetes is controlled, and on doctor recommendations, the HbA1c test may be measured 2 to 4 times each year. The American Diabetes Association recommends HbA1c testing in diabetics at least twice a year. When someone is first diagnosed with diabetes or if control is not good, HbA1c may be ordered more frequently.

Note: If a person has anemia, few type of hemoglobinopathy, hemolysis, or heavy bleeding, HbA1c test results may be falsely low. If someone is iron-deficient, the HbA1c level may be increased. If a person has had a recent blood transfusion, the HbA1c may be inaccurate and may not accurately reflect glucose control for 2 to 3 months.

****** End Of Report ******

SR.LAB.TECHNICIAN

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DMCI 10558

ICU -1

[redacted]

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(A Unit of AS Kidney Hospital & Stone Centre Pvt. Ltd.)

[redacted]

Date	**19/02/2026**	**Srl No.**	**2**	**Patient Id**	**2602190002**
Name	[redacted]	**ge**	**71 Yrs.**	**Gender**	**Female**
Ref. By	[redacted]	**IPD No.**	**25/0927**	**UHID No.**	**P0005082**

COMPLETE BLOOD COUNT (CBC)

Test Name	Result	Unit	Normal Value
Haemoglobin (Hb)	**7.6**	gm/dl	12.0 - 15.0
Total Leucocyte Count (TLC)	**11,400**	/cumm	4000 - 11000
Differential Leucocyte Count (DLC)			
Neutrophil	**82**	%	40 - 75
Lymphocyte	**13**	%	20 - 45
Eosinophil	03	%	01 - 06
Monocyte	02	%	02 - 10
Basophil	00	%	0 - 01
Red Blood Cell (RBC)	**2.94**	million/µL	3.8 - 4.8
Packed Cell Volume (PCV/HCT)	**25.8**	%	37.0 - 48.0
Mean Corpuscular Volume(MCV)	87.9	fl	81.0 - 99.0
Mean Corpuscular Hemoglobin (MCH)	**25.8**	pg	27.0 - 31.0
MCHC	**29.4**	g/dl	32.0 - 36.0
Platelet Count (Plt Count)	2.36	Lakh	1.50 - 4.50

****** End Of Report ******

[redacted]

SR.LAB.TECHNICIAN

[redacted]

M.B.B.S, M.D
CONSULTANT PATHOLOGIST
DMCI 10558

[redacted]

[redacted]

TRUST | CARE | CURE

(A Unit of AS Kidney Hospital & Stone Centre Pvt. Ltd.)

[redacted]

Date	19/02/2026	Srl No.	4	Patient Id	2602190004
Name	[redacted]ge		71 Yrs.	Gender	Female
Ref. By	[redacted]	IPD No.	25/0927	UHID No.	P0005082

STOOL TEST

Occult Blood	Negative	Negative

****** End Of Report ******

SR.LAB.TECHNICIAN

Dr. VIMLA MOURYA
M.B.B.S, M.D
CONSULTANT PATHOLOGIST
DMCI 10558

Barcode No	: **S10505539**	Lab No	: ACC2602193925
Patient Name	: [redacted]	Reg Date	: 19/Feb/2026 03:46PM
Age/Gender	: 71 Y 0 M 0 D /F	Sample Coll. Date	: 19/Feb/2026 02:48PM
Refered By	: SELF	Sample Rec.Date	: 19/Feb/2026 04:19PM
Ref. Lab/Hosp	:	Report Date	: 19/Feb/2026 08:37PM
Client Code/Name	: [redacted]		
Client Add.	[redacted]		

DEPARTMENT OF .IMMUNO BIOCHEMISTRY-1

Test Name	Result	Unit	Bio. Ref. Range
Folic Acid [Folate]			
Sample Type : Serum			
Folate	9.03	ng/ml	2.6 - 12.2

Electro Chemi Luminescent Immuno Assay

Interpretation :
1. Decreased levels of folic acid are seen in megaloblastic anemia, malnutrition, alcoholism, and inadequate dietary intake are associated with a higher risk of fetal malformations during pregnancy.
2. In patients taking methotrexate therapy, antibodies formed may interfere with the assay.

Comments:

Folate (folic acid) is vital for normal cellular functions and plays an essential role in nucleic acid synthesis, methionine regeneration, shuttling and redox reactions of one-carbon units required for normal metabolism and regulation. Folate deficiency can be caused by decreased nutritional intake, poor absorption of ingested folate in the intestine or increased demand of folate, for example during physical activity or pregnancy. Deficiency of folate can also be a result of liver diseases or impaired folate metabolism due to genetic defects or drug interactions.

Hemolysis may significantly increase folate values due to high concentrations of folate in red blood cells. Therefore, hemolyzed samples are not suitable for use in this assay. Samples for folate determinations should be collected from fasting persons. Samples should not be taken from patients receiving therapy with high biotin doses (i.e. > 5 mg/day) until at least 8 hours following the last biotin administration. For diagnostic purposes, the results should always be assessed in conjunction with RBC folate, the patient's medical history, clinical examination, and other findings.

[redacted]

(Director & Chief Pathologist)
Reg. No. DMC-53016

[redacted]

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(A Unit of AS Kidney Hospital & Stone Centre Pvt. Ltd.)

[redacted]

Barcode No	: **S10505539**	Lab No	: ACC2602193925
Patient Name	: [redacted]	Reg Date	: 19/Feb/2026 03:46PM
Age/Gender	: 71 Y 0 M 0 D /F	Sample Coll. Date	: 19/Feb/2026 02:48PM
Refered By	: SELF	Sample Rec.Date	: 19/Feb/2026 04:19PM
Ref. Lab/Hosp	:	Report Date	: 19/Feb/2026 06:30PM
Client Code/Name	: [redacted]		
Client Add.	: [redacted]		

DEPARTMENT OF .IMMUNO BIOCHEMISTRY-1

Test Name	Result	Unit	Bio. Ref. Range
Iron Panel Basic			
Sample Type : Serum			
Iron FerroZine without deproteinization	**18.20**	ug/dl	33-193
UIBC Biochemical	399	ug/dL	63 - 433
TIBC Biochemical	**417.2**	ug/dL	250-400
Transferrin Saturation Calculated	**4.36**	%	15-55

COMMENT:

Serum iron measures the amount of circulating iron that is bound to transferrin. Clinicians order this laboratory test when they are concerned about iron deficiency, which can cause anemia and other problems.

Total iron-binding capacity The test measures the extent to which iron-binding sites in the serum can be saturated. Because the iron-binding sites in the serum are almost entirely dependent on circulating transferrin, this is really an indirect measurement of the amount of transferrin in the blood. Taken together with serum iron and percent transferrin saturation clinicians usually perform this test when they are concerned about anemia, iron deficiency or iron deficiency anemia. However, because the liver produces transferrin, liver function must be considered when performing this test. It can also be an indirect test of liver function, but is rarely used for this purpose.

[redacted]

(Director & Chief Pathologist)

Reg. No. DMC-53016

This test was performed:Accuprobe Diagnostics:B-14/7,Jhilmil Industrial Area,Delhi-110095, M-9599593622

[redacted]

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[redacted]

Barcode No	: **S10505539**	Lab No	: ACC2602193925
Patient Name	: [redacted]	Reg Date	: 19/Feb/2026 03:46PM
Age/Gender	: 71 Y 0 M 0 D /F	Sample Coll. Date	: 19/Feb/2026 02:48PM
Refered By	: SELF	Sample Rec.Date	: 19/Feb/2026 04:19PM
Ref. Lab/Hosp	:	Report Date	: 19/Feb/2026 07:09PM
Client Code/Name	: [redacted]		
Client Add.	:		

DEPARTMENT OF .IMMUNO BIOCHEMISTRY-1

Test Name	Result	Unit	Bio. Ref. Range
Vitamin B12 (Cynocobalamin)			
Sample Type : Serum			
Vitamin B12 Level Electro Chemi Luminescent Immuno Assay	310.00	pg/ml	197-711

Comment:

Vitamin B12 (cobalamin) is an important water-soluble vitamin. In contrast to other water-soluble vitamins it is not excreted quickly in the urine, but rather accumulates and is stored in the liver, kidney and other body tissues. Humans obtain Vitamin B12 exclusively from animal dietary sources, such as meat, eggs and milk. As a result, a vitamin B12 deficiency may not manifest itself until after 5 or 6 years of a diet supplying inadequate amounts. Vitamin B12 functions as a methyl donor and works with folic acid in the synthesis of DNA and red blood cells and is vitally important in maintaining the health of the insulation sheath (myelin sheath) that surrounds nerve cells. Preservatives such as fluorides & ascorbic acid interfere with this assay. Excessive exposure of the specimen to light may alter Vitamin B12 result.. **Kindly correlate with clinical conditions.**

[redacted]

(Director & Chief Pathologist)

Reg. No. DMC-53016

This test was performed:Accuprobe [redacted]

[redacted]

[redacted]

[redacted]

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[redacted]

[redacted]

Date	**19/02/2026**	**Srl No.**	**2**	**Patient Id**	**2602190002**
Name	[redacted]	**ge**	**71 Yrs.**	**Gender**	**Female**
Ref. By	[redacted]	**IPD No.**	**25/0927**	**UHID No.**	**P0005082**

COMPLETE BLOOD COUNT (CBC)

Test Name	Result	Unit	Normal Value
Haemoglobin (Hb)	**7.6**	gm/dl	12.0 - 15.0
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Lymphocyte	**13**	%	20 - 45
Eosinophil	03	%	01 - 06
Monocyte	02	%	02 - 10
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MCHC	**29.4**	g/dl	32.0 - 36.0
Platelet Count (Plt Count)	2.36	Lakh	1.50 - 4.50

****** End Of Report ******

[redacted]

SR.LAB.TECHNICIAN

[redacted]

[redacted]

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